



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رابعياً : هويه عبدالعال فرعلي حماده

المحافظة : 6 أكتوبر الحي / المدينة : الشيخ زايد

رقم البطاقة : 26402192500868 رقم التليفون : 01090736879

العمر (تاريخ الميلاد) : 19-02-1964 المهنة : ربه منزل

العنوان : اشعه

تاريخ تسجيل الإستمارة : 30-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعة مقطعية على الصدر والبطن والحوض

[11129]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /د. /د. /د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: **عبد الجليل فرطى حمادو**
National ID: **35402192500868**



تقرير طبي

المريض

تحتية طبية وبعد

بشأن تقرير طبي للمريض

إمضاء الطبيب المعتمد من

رقم طبي:

بسم:

تتصرف بأن أوضح إمضاء ياتي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الشغل :
+ 0000 yyyymmdd

وبعد الفشل والموصات والمتابعة وجدته

المرحلة إمكاني من يوم غرضه والشيء و يحتاج الى CT scan of the chest, abdomen and pelvis.

توقيع :

دكتور محمد حمادو

التاريخ :

05-07-2026

الطبيب المعالج :
الاستاذ الدكتور



 جمهورية مصر العربية
 وزارة الداخلية
 إدارة المرور
 رخصة قيادة
 رقم الرخصة: ٢٦٤٠٢١٩٢٥٠٠٨٦٨
 تاريخ الميلاد: ٢٦/٠٨/١٩٨٠
 رقم الهوية: ٢٦٤٠٢١٩٢٥٠٠٨٦٨
 رقم الترخيص: ٢٦٤٠٢١٩٢٥٠٠٨٦٨



 جمهورية مصر العربية
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 تاريخ الميلاد: ٢٦/٠٨/١٩٨٠
 رقم الهوية: ٢٦٤٠٢١٩٢٥٠٠٨٦٨
 رقم الترخيص: ٢٦٤٠٢١٩٢٥٠٠٨٦٨

Name : Hawiya Abdel Aal Farghaly Hamada
 Branch : Dokki
 Ref.Physician Prof : Goustav Roussey Egypt

Date : 06-07-2026 07:50
 Medical No. : 409947

Whole-body skeletal scintigraphy report

Clinical History: 62-year-old female patient with a history of right breast cancer (post-mastectomy, chemotherapy, and radiotherapy in 2012) presents for follow-up.

Technique: Anterior/posterior whole-body planar imaging acquired post-IV injection of Tc-99m MDP

Findings

Compared to historical reports (latest March 2025), the current study demonstrates a highly extensive, widespread increase in focal radiotracer uptake throughout the axial and appendicular skeleton, indicating marked disease progression:

- **Skull:** Multiple prominent foci of intense uptake scattered throughout the calvarium.
- **Axial Skeleton:** Intense, multi-level focal uptake throughout the cervical, thoracic, and lumbar spine. Persistent intense uptake is noted in the sternum and multiple ribs bilaterally.
- **Pelvis & Lower Limbs:** Widespread, confluent uptake involving both iliac bones, the sacrum, and the bilateral ischiopubic bones, with progressive accumulation in both acetabular regions and proximal femora (including the right greater trochanter).
- **High bone-to-background ratio.** Both kidneys are faintly visualized.

Conclusion

- Widespread, extensive osteoblastic skeletal metastases involving the calvarium, entire spine, sternum, bilateral ribs, pelvis, and proximal femora.
- Compared to prior reports, the findings confirm a **significant progressive course** with an increased skeletal tumor burden.

Much oblige
 Dr. Alaa Nagi, MD
 Nuclear medicine



Patient's Name: Hawya Abdel-Aal Fergaly Hamada
Date: 2026-06-03
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

As compared to previous study dated 2/11/2025, the current study revealed

I- CHEST:

- o Post right mastectomy status
- o still seen tiny right middle lobar nodule subpleural in location measuring 3 mm seen in the lateral segment.
- o No consolidation or collapse.
- o Still seen prominent mediastinal lymph nodes
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.
- o Still seen multiple lytic and sclerotic osseous lesions seen in right femoral neck, bilateral iliac bones, dorsal and lumbar vertebral bodies as well as the sternum

Opinion:

- ❖ **Known case of right breast cancer showing overall stationary course**

MDT FORM

30 / 11 / 2025 G

Name	عبد العال فرناي	MRN	1303/2025
Age	61	ID	26402132500868
Menopausal status	post	Phone	01221232333
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: Left breast cancer metastatic to bone

ER: 4/8

PR: 4/8

HER 2: -ve

KI67: Low

Sonomammography:

Mets Workup:

hx Lt BCS 2013

TAM for 9 yrs 2022

3/24 → bone mets, started femara till now

BS → 3/24: pelvis, skull, D7, L1, rt iliac bone extending to ischial bone,

Recommendations: - rt femoral head and trochanteric area

BS → 3/25: Newly developed lesion in sternum with mixed response

BS → 10/13/25: Newly developed lesion at skull, left scapula, right 10th rib, sternum and multiple dorso-lumbar vertebrae

CT ~~test~~ CAP → 2/11/25: Multiple axial and appendicular metastatic lytic and sclerotic lesion

MDT OUTCOME & PLAN

Dr. [Signature] for CDK16 + doxorubicin + Zol 9.5mg

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

Nutrition

Psychiatric

[Signature]

[Signature]



Patient's Name: Hawya Abdel-Aal Fergaly Hamada
Date: 2026-06-03
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

I- CHEST:

- Post right mastectomy status
- Tiny right middle lobar nodule subpleural in location measuring 3 mm seen in the lateral segment.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulacae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple sclerotic osseous lesions seen in right femoral neck, bilateral iliac bones, dorsal and lumbar vertebral bodies as well as the sternum

Opinion:

- ❖ Known case of right breast cancer showed right breast nodular lesion as well as right axillary lymph nodes.
- ❖ Multiple osseous metastatic lesions.
- ❖ Right middle lobar tiny pulmonary nodule.

Dr. Asser Ahmed

Patient Name: Hwih Abd Al Aal Farghali	Date : 02/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 61 Year(s)
Medical ID: 1309/2025	Pat. ID: 132748

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION: -

Clinical History:

- History of right MRM.

Findings:

- Clear right mastectomy bed with no residual or recurrent lesions.
- Clear left breast.
- No suspicious axillary lymph nodes.
- Right apical tiny pulmonary nodule measuring 2mm.
- Clear other both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- Few enlarged mediastinal lymph nodes at retrocaval and carinal group largest measuring 13x9mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Clear right mastectomy bed with no residual or recurrent lesions.
- Clear left breast.
- No suspicious axillary lymph nodes.
- Right apical tiny pulmonary nodule for follow up.
- Enlarged mediastinal lymph nodes.

MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION: -

Findings:

- The liver is of average size 15cm with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.

Patient Name: Hwih Abd Al Aal Farghali	Date : 02/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 61 Year(s)
Medical ID: 1309/2025	Pat. ID: 132748

- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size & normal parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs including uterus.
- No adenaxal cysts or masses.
- No abdominal or pelvic masses or collections.
- Paraumbilical hernia is seen with a defect measuring 3cm with fat content.
- Multiple axial and appendicular skeleton lytic and sclerotic lesions are seen at cervical, dorsal, lumbar vertebrae as well as right iliac bone and right femoral head denoting metastasis.

Opinion:

- Paraumbilical hernia with fat content.
- Multiple axial and appendicular metastatic lytic and sclerotic lesions.

Dr. Mai Shaaban MD

Dr. Mai Shaaban

Dr. Mai Shaaban

Signature Dr. Mohamed Hassan
04/11/2025 3:13 AM

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 5172

Name هويدا عبد العال فرغلي

Age: 49

Sex: female

Clinical diagnosis BREAST MASS

Hospital no: 14164

Nature of specimen COMPLETION MASTECTOMY

Department: 27a

Date: 04/11/2013

PATHOLOGY REPORT

GROSS:

Mastectomy specimen with overlying skin ellipse measures 14x4 cm showing a sutured wound measures 2.5 cm long. Sections revealed firm greyish area measures 1x1 cm surrounding the sutures situated 1 cm from skin & 2 cm from deep margin. Another greyish area, rubbery, measures 0.5x0.5 cm was detected.

Microscopic:

Examination of the specimen received revealed fibrosis, fat necrosis & giant cells at site of previous resection of the tumor.

Rest of breast tissue shows an area of fibroadenosis with ductal atypical epithelial hyperplasia & focal solid intraductal carcinoma.

Nipple & deep margins are free.

No evidence of invasive malignancy.

DIAGNOSIS:

* COMPLETION MASTECTOMY:

* FOCAL SOLID INTRADUCT CARCINOMA (<5%)

* FIBROADENOSIS WITH ATYPICAL EPITHELIAL HYPERPLASIA

* FAT NECROSIS.

* FREE NIPPLE & DEEP MARGINS.

PROF. DR. SAHAR TALAAT
DR. MONA SALAH

د. سحر تالاات
د. منى صلاح



د. سحر تالات

وحدة دراسات الأورام
IMMUNOHISTOCHEMISTRY
٠١٠٦٨٥٢٩٦٨٤ : ٢

Patient Name: د. سحر تالات

lab.No:2013/899

Date: 12/2013

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections from paraffin blocks were treated with monoclonal antibodies against ER, PR, Her 2 neu and Ki67 using streptavidin biotin peroxidase system. DAB was used as substrate as well as chromogen. Hematoxylin was used as counter stain

Results:

- Examination of sections treated for ER and PR revealed moderately positive nuclear staining of tumor cells by chromogen.
- Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen
- Examination of sections treated for Ki67 revealed positive nuclear staining in < 14 % of tumor cells by chromogen.

DIAGNOSIS:

- POSITIVE FOR ER AND PR SCORE 4/8
- NEGATIVE FOR HER 2 NEU
- LOW PROLIFERATION INDEX

PROF. DR. SAHAR TALAAT

Patient Name: هوية عبد المالح فرطلي	Patient ID: 26402192500868
Age: 57 Years	Sex: F

WOMEN IMAGING DEPARTMENT

EEFI Sided Full Field Mammography with Complementary Ultrasound Examinations Revealed

Indications

- *Right mastectomy.*

Findings:

- Scattered fibroglandular densities of the breasts (ACR 'b').
- No spiculated mass lesions or microcalcific clusters seen.
- Complementary Ultrasound
- Clear operative bed with no sizable residual/recurrent masses.
- Left breast shows no underlying solid or cystic lesions.
- Normal left sided skin thickness and contour.
- Normal left breast nipple areola complex.
- No pathologically enlarged right axillary, infra or supraclavicular lymph nodes.
- Non specific left axillary lymph nodes.

Opinion:

- Status post right mastectomy showing clear operative bed with no evidence of sizable residual/recurrent masses.
- Normal study of the Left breast (BIRADS I).

Thanks for reference

Dr. Safar A. Alsh

Dr. Mahmoud Mekhaimar, MSc

Patient name: HowedaSaed

Date: 22/10/2022

Report

non-contrast CT examination of the Chest revealed:

- The examination is degraded by the patient respiratory movement.
- Evidence of right breast lesion.
- Left upper lobar two pulmonary nodules are noted (7.5 & 6 mm).
- Bilateral diffuse patchy ground glass veiling and multiple atelectatic bands are noted ?infectious. For clinio-laboratory correlation.
- Clear both costo-phrenic recesses.
- Normal cardiac size with no pericardial effusion.
- No mediastinal masses or lymphadenopathy are detected.
- ✍ Multipylelytic bone deposits in the examined skeleton for bone scan.

Dr. Aya Mohamed

Name : Hawiya Abdel Aal Farghali
Branch : Dokki
Ref. Physician Prof : Gostaf Rosen Hospital

Date : 10-09-2025 02:32
Medical No. : 199178

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- 61-year-old lady, with history of right breast cancer, referred for follow up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

Considering the prior bone scan dated 26/02/2022:

- Newly-developed active osseous lesions at the skull, left scapula, right 10th rib, sternum and multiple dorso-lumbar vertebrae.
- Still seen active lesion at the right acetabulum.
- Degenerative changes are seen at the spine and big joints.

Impression:

- Multiple metastatic osseous lesions as described.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

ECHO REPORT

NAME: طارق عبد العالي فؤاد DATE: 7 / 9 / 2026
ID NUMBER: / SEX: ♀ AGE: 61y.

EF	67.1.	55-75%	PWT	0.8	0.6-1.1cm
LVEDD	3.9	3.5-5.8cm	AO.R	2.0	3.3-7cm
LVEDS		2-4cm	LA	3.2	2-4cm
FS		25-45%	RV		0.7-2.3cm(supine)
SWT	0.9	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions normal
LV Wall Thickness normal
LV Diastolic Function DD grade I
SWMA 0 SWMA
LA and Ao Root Dimensions normal
Mitral Valve trivial MR
Aortic Valve severe AV with no sig. systolic gradient
Tricuspid Valve trivial TR
Pulmonary Valve normal
Right Side and Systolic Function normal
Intracardiac Masses and Effusion 0

DR.

John

06/05/2025

MDT FORM

Name:	Hanna Abd Elazab Faragalla	PHEN:	4052
Age:	63 Y	ID:	
Menopausal Sta		Phone:	0128516533
NRS SCORE:		Marital Status:	
BMI:		Family History:	
Gender:		Reason OF MDT:	
CO Morbidity:		OP	
Surgical History:	MHM	ECOG Performance Status:	
		☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4	

Clinical Data:

BXI:	
ER:	PR: HER2: KI67:

History:

LI MHM 2013, for 4/8 PR 4/8 her2 0 RECEIVED ADJUVANT CHEMO+ PORT
 + Tamoxifen for 9y TILL 2022 9/2024: bone meta w/ femora 9/2024 till
 now RS 9/2024: pelvic, skull, D7, L1, L4 flat bone extending to ischial
 bone, rt femoral head and trochanteric area RS 3/2025: newly developed
 lesion in sternum with mixed response RS 10/9/2025: REVEL DEVELOPED
 LESION at skull, left scapula, right 10th rib, sternum and multiple dorsi-
 lumbar vertebrae
 pt received fasludex , ibranse *3 cycles

bone scan 6-7-2025 >>> DP
 CT CAP 3-6-2026 >>> SD



T N M

Right Side			Left Side		
Q1	Q2	Q3	Q4	Q1	Q2

Clinical Staging:

Pathological Staging:

Immunohistochemistry:

Biomarker/Molecular:

Metastasis:

Recommendations:

MDT Outcome & Plan:

for re-biopsy from breast lesion , BRCA test , AKT
 meanwhile at Tazol weekly

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
Dr. Hanaa Farag			

Name : Hawiyya Abdel Aal Farghaly Hamada
Branch : Dokki
Ref/Physician Prof: Goustav Roussey, Egypt

Date : 06-07-2026 07:50
Medical No. : 409947

2025-07-06 07:50
409947

Whole-body skeletal scintigraphy report

Clinical History: 62-year-old female patient with a history of right breast cancer (post-mastectomy, chemotherapy, and radiotherapy in 2012) presents for follow-up.

Technique: Anterior/posterior whole-body planar imaging acquired post-IV injection of Tc-99m MDP

Findings

Compared to historical reports (latest March 2025), the current study demonstrates a highly extensive, widespread increase in focal radiotracer uptake throughout the axial and appendicular skeleton, indicating marked disease progression:

- **Skull:** Multiple prominent foci of intense uptake scattered throughout the calvarium.
- **Axial Skeleton:** Intense, multi-level focal uptake throughout the cervical, thoracic, and lumbar spine. Persistent intense uptake is noted in the sternum and multiple ribs bilaterally.
- **Pelvis & Lower Limbs:** Widespread, confluent uptake involving both iliac bones, the sacrum, and the bilateral ischiopubic bones, with progressive accumulation in both acetabular regions and proximal femora (including the right greater trochanter).
- High bone-to-background ratio. Both kidneys are faintly visualized.

Conclusion

- Widespread, extensive osteoblastic skeletal metastases involving the calvarium, entire spine, sternum, bilateral ribs, pelvis, and proximal femora.
- Compared to prior reports, the findings confirm a significant progressive course with an increased skeletal tumor burden.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient's Name: Hawya Abdel-Aal Fergaly Hamada
Date: 2026-06-03
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

As compared to previous study dated 2/11/2025, the current study revealed

I- CHEST:

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- o still seen tiny right middle lobar nodule subpleural in location measuring 3 mm seen in the lateral segment.
- o No consolidation or collapse.
- o Still seen prominent mediastinal lymph nodes
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- o The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- o Normal appearance of the pelvic organs with no pelvic masses or collections.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.
- o Still seen multiple lytic and sclerotic osseous lesions seen in right femoral neck, bilateral iliac bones, dorsal and lumbar vertebral bodies as well as the sternum

Opinion:

❖ Known case of right breast cancer showing overall stationary course



١٩٤٠٢١٩٢٥٠٠٨٦٨
الاسم

عبدالعال فرغش حماده

الاسم

١٩٤٠٢١٩٢٥٠٠٨٦٨

الاسم

مصلحة الأحوال المدنية

١٩٤٠٢١٩٢٥٠٠٨٦٨

Patient Name: Hwib Abd Al Aal Farghali	Date : 02/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 61 Year(s)
Medical ID: 1309/2025	Pat ID: 132746

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH
CORONAL RECONSTRUCTION:-**

Clinical History:

- History of right MRM.

Findings:

- > Clear right mastectomy bed with no residual or recurrent lesions.
- > Clear left breast.
- > No suspicious axillary lymph nodes.
- > Right apical tiny pulmonary nodule measuring 2mm.
- > Clear other both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- > Few enlarged mediastinal lymph nodes at retrocaval and carinal group largest measuring 13x9mm.
- > No evidence of pleural effusion, pleural thickening or masses seen.
- > Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- > Normal CT appearance of the thoracic aorta and the great vessels.

OPINION:

- Clear right mastectomy bed with no residual or recurrent lesions.
- Clear left breast.
- No suspicious axillary lymph nodes.
- Right apical tiny pulmonary nodule for follow up.
- Enlarged mediastinal lymph nodes.

**MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V.
CONTRAST ADMINISTRATION:-**

Findings:

- > The liver is of average size 15cm with homogenous CT texture. No focal lesions or contour irregularities.
- > Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.

Patient Name: Hwib Abd Al Aai Farqhal	Date : 02/11/2025
Exam : CT C&P+C (CT Chest Abd Pelvis)	Age : 61 Year(s)
Medical ID: 1309/2025	Pat. ID: 132746

- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Average size & normal parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suparenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs including uterus.
- No adnexal cysts or masses.
- No abdominal or pelvic masses or collections.
- Paraumbilical hernia is seen with a defect measuring 3cm with fat content.
- Multiple axial and appendicular skeleton lytic and sclerotic lesions are seen at cervical, dorsal, lumbar vertebrae as well as right iliac bone and right femoral head denoting metastasis.

Opinion:

- Paraumbilical hernia with fat content.
- Multiple axial and appendicular metastatic lytic and sclerotic lesions.

Dr. Mai Shaaban MD

N

Dr. Mai Shaaban

Dr. Mai Shaaban

Signature Dr. Mohamed Hassan
04/11/2025 3:13 AM

MDT FORM

30 / 11 / 2025 6

Name	مروة عبد الحك فؤاد	MRN	1303/2025
Age	61	ID	2640242500868
Menopausal status	Post	Phone	01224292433
NRS SCORE		Marital status	
BMI		Family history	
CO morbidity		REASON OF MDT	

Clinical Data

BX: Left breast cancer metastatic to bone

R

L

ER: 4/8 PR: 4/8 HER 2: -ve Ki67: Low

Sonomammography:



Mets Workup:

T N M

hx Lt BCs 2013
TAM for 9 yrs 2022

5/24 → bone mets, started femara till now
BS → 5/24, Pelvis, skull, D, L, T, iliac bone extending to ischial bone,

Recommendations: CT femoral head and tracheatic area

BS → 3/25: Newly developed lesion in sternum with mixed response

BS → 5/25: Newly developed lesion at skull, left scapula, right 10th ribs,
sternum and multiple dorso-lumbar vertebrae

CT ~~chest~~ → 2/11/25: Multiple axial and appendicular metastatic lytic and sclerotic
CAP lesions

MDT OUTCOME & PLAN

Guid to COlell + Gholader + Zoh g8w
20/11/25

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

20/11/25



MDT form

1- Patient ID:

1- Name: *Shamshul Hossain Khan*

Sex:

3- Medical number:
Female

83274887
Old Med:

Date of birth:
1309-2028

2- ID number: 20402102500566

Patient Age: 61 Y



Family history

History mother

Phone

Home
01221 794923

BMI

Wt

REASON OF MDT

Reason for Ref

Current status

Not on treatment

REC Score

Yes/No

SCOUT

LCIS

DATE OF MDT

Date of Ref
2025-03-18 CT

Current status

Not on treatment

Management status

Management status

Initial stage

Initial stage

Clinical data

Breast Imaging:

Melt's Workup:

DX

ER

4/8

PR

4/8

Her 2 Neu

SISS

K167

Immunohistochemistry

Immunohistochemistry

Immunohistochemistry

Immunohistochemistry

Immunohistochemistry

T

N

M

BONE

Sonohammography

PATIENT HISTORY

breast cancer since 2013-->lumpectomy -->completion mastectomy-->DCIS
adj chemo AC x4-->4 taxanes-->PORT-->adj endocrine x 9 yrs end on 2022
bony mets 2024-->AI-->now DP (only bone)-->palliative RTH

MDT OUTCOME & PLAN

BONE SCAN ONLY COMPARED WITHOUT IMAGES
OLD BIOPSY NOT AVAILABLE
FOR BONE SCAN AFTER 3 MONTH WITH COMPARISON LAST STUDY
KEEP ON AI, ZOMETA/3 MONTH TILL RESULT

SIGNATURE

RADIOLOGY

Signature

SURGERY

Signature

PATHOLOGY

Signature

Psychiatric

Signature

Nutrition

Signature

Pat.Name: **عبدية عبدالعزى لطفى**

Pat.ID: 442526

Date: 13/08/2024

Pat. Age 60 Year(s)

Ref.Prof.Doc: Mohamed Saad Al Emam

MRI OF THE PELVIS & BOTH HIPPS

TECHNIQUE:

- Sagittal T2WIs
- Axial T1 and T2 WI
- Coronal T1, T2 and STIR WI

FINDINGS:

- Multiple variable sized extensive infiltrative marrow lesions are seen involving the right hemi pelvis, right femoral head and neck. The most sizable lesion involves most of the right iliac blade extending inferiorly to the acetabulum, ischium, superior and inferior pubic rami. It appears associated with minimal extra osseous soft tissue rim on non contrast basis. Other smaller lesions are seen at the right sacral alum as well as the right femoral head and neck. Fewer smaller similar lesions affecting the iliac bone and acetabulum. All these lesions display low signal in T1 WIs and bright signal in T2 and STIR WIs.
- Intact both hip joints with no joint effusion or synovial thickening.
- Normal signal and comparable bulk of the different muscle groups around both hip joints.
- Preserved fascial fat planes.

OPINION:

- Multiple extensive infiltrative marrow lesions involving the sacrum, iliac bones and right femur impressive of metastatic deposits.

Dr.Dafaa Ibrahim, MD

Dr. Mafmoud ElFekky.





Patient name: Hawea Abd Alaal

Study Date: 22/10/2022

CT scan of the abdomen & pelvis revealed:

****Reported case of invasive right breast cancer.**

****The examination is partially degraded by motion artifact, absence of adequate thin cutting and triphasic post contrast enhancement**

- Average hepatic size showing homogenous parenchymal pattern. No focal hepatic lesions or biliary radicles dilatation.
- **Relatively bulky left suprarenal gland.**
- Normal CT appearance of the spleen, pancreas, right suprarenal gland, IVC and urinary bladder. **Splenule is noted anterior to the spleen.**
- Normal appearance of both kidneys with no gross backpressure changes or radiodense calculi.
- **Aortic calcified atheromatous plaques are noted.**
- **Few enlarged paraaortic, mesenteric and aorto-caval lymph nodes, the largest is at the latter measuring about 1.2x0.6 cm in maximum axial dimensions.**
- No ascites or pelvic collections.
- **Few pelvic phlebolith are also noted.**

Dr.Aya Fawzy, MSc.

Name: هوية عبد العال Age: 60 Ys Date: 17 August 2024

Pelvi-abdomenel U/S study, revealed

- Liver is average in size, measuring about 13.8 cm, with smooth outline & diffuse bright homogenous echopattern. No focal lesions were detected.
- No intra or extra hepatic biliary radicles dilatation.
- Gall bladder: is normal capacity and wall thickness. No stones or mud inside.
- Portal vein is patent & measures (9.6 mm) and C.B.D (0.4 mm).
- Spleen normal shape & measures (8.8 cm) and homogenous echogenicity. No focal.
- Pancreas & Para-Aortic area: normal U/S appearance.
- Both kidneys: They are of normal in size, size, shape, parenchymal thickness and echogenicity with no renal stones, no back pressure obstructive changes.
 RK = 10.3 X 3.7 cm with parenchymal thickness = 1.5 cm
 LK = 10.4 X 3.7 cm with parenchymal thickness = 1.0 cm
- Urinary bladder: is empty at time of examination.
- Uterus: is of average size & homogenous myometrium.
- Adenexa: no gross focal lesions.
- No ascites or pelvic collection.
- Prominent colonic gases.

Conclusion:

- Bright liver (?? Fatty infiltration).
- Prominent colonic gaseous distention.

 Much obliged, yours,
Akram Hafez
 Radiodiagnosis Specialist

Name: Mrs. Hawia Abdel-Aal Farghaly

Date: 24/10/2022

Referring Prof. Dr.: Ehab Diaa.

ID: 2222063987

Acc.: 4713151

DXA Bone Densitometry

Technique:

Your patient completed a BMD and morphometry test for the lumbar spine, femoral neck and distal end forearm using lunar prodigy DXA system. The following summarizes the results of our evaluation.

	Region	BMD (g/cm ³)	T-Score	WHO classifications
Spine	L1-L4	0.840	-2.9	Osteoporosis
	Left forearm	Radius 33%	-4.1	Osteoporosis
Advanced Hip analysis	Total	0.805	-1.6	Osteopenia

WHO criteria for postmenopausal.

Normal
Osteopenia
Osteoporosis

T-Score at or above -1 SD
T-Score between -1 & -2.5 SD
T-Score at or below -2.5 SD

Densitometry Results:

The BMD measured at Forearm Radius 33% is 0.521 g/cm³ with a T-score of -4.1. This patient is considered osteoporotic according to World Health Organization (WHO) criteria.

Fracture Risk Results:

The patient had 11.6% incidence of fracture within 10 years at any site and about 2.1% risk fracture at the hip region compared to 8.1% and <1.0% in an individual of the same age and sex. Fracture risk is high.

Truly yours

Prof. Dr. Haney Ahmad Sami



Name: HAWYA ABDUL AAL FARGHALL	Date: 4/14/2025
Age:	2:27:50 PM
	ID: 500868

Post-contrast CT scan of the abdomen and pelvis revealed:

Reported history of breast cancer showing

- Average sized liver showing homogeneous texture. No focal lesions or signs of intrahepatic biliary ductile dilatation.
- Gall bladder is averagely outlined
- Normal CT appearance of the spleen, showing homogeneous texture. No focal lesions seen with small splenule close to its upper pole.
- Normal form and size of the suprarenal glands, no retroperitoneal masses.
- Normal size and size of both kidneys. No renal stones, masses or back pressure changes.
- No pancreatic masses, also no dilatation of the pancreatic duct.
- No sizable abdominal, retroperitoneal or pelvic lymphadenopathy.
- No ascites.
- Normal pelvic organs
- Well distended urinary bladder showing regular outline, normal wall thickness, with no masses or diverticulum formation.
- Right acetabular and right femoral head heterogeneous focal sclerotic ? metastatic for bone scan correlation

Dr. Asmaa Yafia

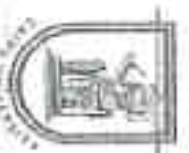
Dr. Marwa Shaker, MD.

Kasr El-Shay Center of Radiation Oncology and
Nuclear Medicine

Faculty of Medicine, Cairo University

"HEMADOK"

Radionuclide Therapy



مركز قصر الشاي

العلاج الإشعاعي والنووي
كلية الطب - جامعة القاهرة

(مركز)

Report

Patient Name	Iwesa Abd el-Aal Farahaly Hamada	Patient Gender	Female
Patient ID	Q44392	DATE	3/25/2025 2:03:02 PM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

Compared to the previous study dated 30/09/2024 (only report is available but no films are available), the current study reveals:

- Newly developed osseous lesion in the sternum.
- Stationary course of the previously noted osseous tracer uptake related to the skull and DV7, LV1.
- Progressive course of the previously noted osseous tracer uptake related to right acetabulum and greater trochanter.
- Regressive course of the previously noted tracer uptake related to the right iliac bone.
- Still non-visualized right kidney for radiological correlation.

Opinion

- Follow-up study showing newly developed lesion in the sternum with mixed response of the previously seen osseous lesions, for follow up.

Dr. Taysaf Hesham

Dr. Nahla Dessoki MD

Patient Name: HAWEAA A ELAAL FARGARY	Patient ID: 26402192500868
Study Date: 26/01/2022	Age: 57 Years

Left Sided Full Field Mammography with Complementary Ultrasound
Examinations Performed

Indications:

- Right mastectomy

Findings:

- Scattered fibroglandular densities (ACR'b').
- No spiculated mass lesions or microcalcific clusters seen.
- Complementary Ultrasound showed no underlying solid or cystic lesions.
- Examination of the operative bed showed no residual or recurrent mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex.
- left non specific axillary lymphadenopathy.
- no supracavicular lymphadenopathy

Opinion:

- Clear operative bed (BIRADS 2).
- Normal sono mammography study of the left breast (BIRADS 1).

Thanks for reference

Dr. Nourhan Mohamed

Women Imaging

Date & Time: 26/01/2022 11:04:19

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 5172

Name: عبد القادر طي

Age: 49 Sex: female

Clinical diagnosis: BREAST MASS

Hospital no: 14164

Nature of specimen: COMPLETION MASTECTOMY

Department: 27a

Date: 04/11/2013

PATHOLOGY REPORT

GROSS:

Mastectomy specimen with overlying skin ellipse measures 14x4 cm showing a sutured wound measures 2.5 cm long. Sections revealed firm greyish area measures 1x1 cm surrounding the sutures situated 1 cm from skin & 2 cm from deep margin. Another greyish area, rubbery, measures 0.5x0.5 cm was detected.

Microscopic:

Examination of the specimen received revealed fibrosis, fat necrosis & giant cells at site of previous resection of the tumor.
Rest of breast tissue shows an area of fibroadenosis with ductal atypical epithelial hyperplasia & focal solid intraductal carcinoma.
Nipple & deep margins are free.
No evidence of invasive malignancy.

DIAGNOSIS:

- * COMPLETION MASTECTOMY:
- * FOCAL SOLID INTRADUCT CARCINOMA (<5%)
- * FIBROADENOSIS WITH ATYPICAL EPITHELIAL HYPERPLASIA
- * FAT NECROSIS.
- * FREE NIPPLE & DEEP MARGINS.

PROF. DR. SAHAR TALAAT
DR. MONA SALAH

د. سحر تالات
د. منة صالح



Name: HAWYA ABDELAAL FARGHALI	Date: 4/7/2025
Age:	ID: 500868

THORACIC IMAGING UNIT

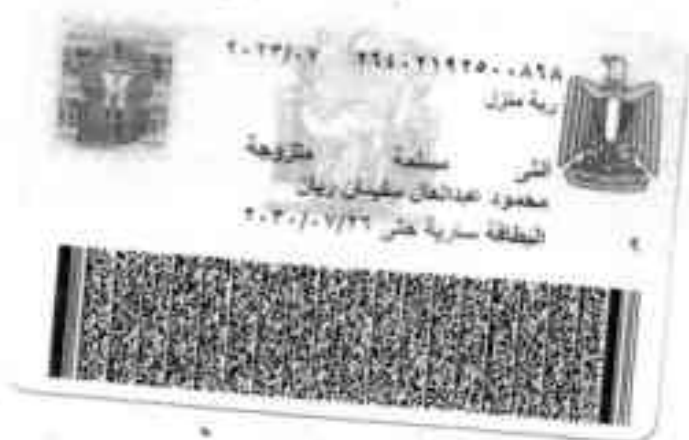
CT SCAN OF THE CHEST REVEALED:-

- Evidence of right mastectomy.
- No pulmonary consolidation or cavitation.
- Right lower lobe few tiny 2 mm pulmonary nodules.
- Subcentimetric mediastinal, right hilar and left axillary lymph nodes.
- No pleural or pericardial sac collections.
- No gross cardiomegaly.
- Scattered vertebral and sternal heterogenous osseous lesions, for bone scan correlation.

Opinion:

- No current CT evidence of pneumonia.
- Right lower lobe few tiny pulmonary nodules, for follow-up.

Dr. Moustafa Khairy, MD





Name: هوبة عبد العال Age: 60 Ys Date: 17 August 2024

Pelvi-abdomenal U/S study, revealed

- Liver is average in size, measuring about 13.8 cm, with smooth outline & diffuse bright homogenous echopattern. No focal lesions were detected.
- No intra or extra hepatic biliary radicles dilatation.
- Gall bladder: is normal capacity and wall thickness. No stones or mud inside.
- Portal vein is patent & measures (9.6 mm) and C.B.D (0.4 mm).
- Spleen normal shape & measures (8.8 cm) and homogenous echogenicity. No focal.
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RK = 10.3 X 3.7 cm with parenchymal thickness = 1.5 cm
LK = 10.4 X 3.7 cm with parenchymal thickness = 1.0 cm
- Urinary bladder is empty at time of examination.
- Uterus: is of average size & homogenous myometrium.
- Adenexa: no gross focal lesions.
- No ascites or pelvic collection.
- Prominent colonic gases.

Conclusion:

- ***Bright liver (?? Fatty infiltration).***
- ***Prominent colonic gaseous distention.***

Much obliged, yours,

Akram Hafez

Radiodiagnosis Specialist

Patient name: Hawea Abd Alaal

Study Date: 22/10/2022

CT scan of the abdomen & pelvis revealed:

****Reported case of invasive right breast cancer.**

****The examination is partially degraded by motion artifact, absence of adequate thin cutting and triphasic post contrast enhancement.**

- Average hepatic size showing homogenous parenchymal pattern. No focal hepatic lesions or biliary radicles dilatation.
- **Relatively bulky left suprarenal gland.**
- Normal CT appearance of the spleen, pancreas, right suprarenal gland, IVC and urinary bladder. **Splenule is noted anterior to the spleen.**
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- **Aortic calcified atheromatous plaques are noted.**
- **Few enlarged paraaortic, mesenteric and aorto-caval lymph nodes, the largest is at the latter measuring about 1.2x0.6 cm in maximum axial dimensions.**
- No ascites or pelvic collections.
- **Few pelvic phleboliths are also noted.**

Dr.Aya Fawzy, MSc.



Name: HAWYA ABDELAAL FARGHALI	Date: 4/7/2025
Age:	ID: 500868

THORACIC IMAGING UNIT

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- No pulmonary consolidation or cavitation.
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- No pleural or pericardial sac collections.
- No gross cardiomegaly.
- Scattered vertebral and sternal heterogenous osseous lesions, for bone scan correlation.

Opinion:

- No current CT evidence of pneumonia.
- Right lower lobe few tiny pulmonary nodules, for follow-up.

Dr. Moustafa Khairy, MD



Name: HAWYA ABDELAAL FARGHALI	Date: 4/14/2025 2:27:50 PM
Age:	ID: 500868

Post-contrast CT scan of the abdomen and pelvis revealed:

****Reported history of breast cancer showing**

- Average sized liver showing homogenous texture. No focal lesions or signs of intrahepatic biliary radicle dilatation.
- Gall bladder is averagely distended
- Normal CT appearance of the spleen, showing homogenous texture. No focal lesions seen with small splenule close to its upper pole.
- Normal form and size of the suprarenal glands, no retroperitoneal masses.
- Normal site and size of both kidneys. No renal stones, masses or back pressure changes.
- No pancreatic masses, also no dilatation of the pancreatic duct.
- No sizable abdominal, retroperitoneal or pelvic lymphadenopathy.
- No ascites.
- Normal pelvic organs
- Well distended urinary bladder showing regular outline, normal wall thickness, with no masses or diverticulum formation.
- Right acetabular and right femoral head heterogeneous focal sclerosis ? metastatic for bone scan correlation

Dr. Asmaa Yahia

Dr. Marwa Shaker, MD.



Inwa Abd el-Aal Farghaly Hamada	Patient ID: Q42591	Female
Q42591	9/30/2024 11:52:55 AM	

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

- Active osseous lesions are noted at following sites: skull bones, mid and DV7, LV1, right iliac bone extending to ischial bone and right femoral head and trochanteric area.
- Osteoarthritic changes of the both knees are seen.

Opinion

- Active metastatic osseous lesions as described above.

Dr. Mohamed AlaaEldin

Prof. Ahmed Kandeel, MD



AL SHROOK SCAN

الشروق للأشعة

15184

Pat.Name: فؤاد عبدالعالم لمرغش

Pat.ID: 442526

Date: 13/08/2024

Pat. Age 60 Year(s)

Ref.Prof.Doc: Mohamed Saad Al Emam

MRI OF THE PELVIS & BOTH HIPS

TECHNIQUE:

- Sagittal T2WIs
- Axial T1 and T2 WI
- Coronal T1, T2 and STIR WI

FINDINGS:

- Multiple variable sized extensive infiltrative marrow lesions are seen involving the right hemi pelvis, right femoral head and neck. The most sizable lesion involves most of the right iliac blade extending inferiorly to the acetabulum, ischium, superior and inferior pubic rami. It appears associated with minimal extra osseous soft tissue rim on non contrast basis. Other smaller lesions are seen at the right sacral alum as well as the right femoral head and neck. Fewer smaller similar lesions affecting the iliac bone and acetabulum. All these lesions display low signal in T1 WIs and bright signal in T2 and STIR WIs.
- Intact both hip joints with no joint effusion or synovial thickening.
- Normal signal and comparable bulk of the different muscle groups around both hip joints.
- Preserved fascial fat planes.

OPINION:

- Multiple extensive infiltrative marrow lesions involving the sacrum, iliac bones and right femur impressive of metastatic deposits.

Dr. Dalia Ibrahim, MD

Dr. Mahmoud ElFekky.



AL SHROOK SCAN

الشروق للأشعة

15184

Pat.Name: موديا عبدالمعطي محمد علي

Pat.ID: 442526

Date: 13/08/2024

Pat. Age 60 Year(s)

Ref.Prof.Doc: Mohamed Saad Al Emam

MRI OF THE LUMBOSACRAL SPINE

TECHNIQUE:

- Sagittal T1 and T2 WIs
- Axial T2 WIs

FINDINGS:

- Straightened lumbar lordosis.
- Normal dimensions of the spinal canal.
- Mild lumbar spondylosis evident by small marginal osteophytic lipping of the vertebral endplates and degeneration of the lumbar discs.
- L3-4, L4-5 and L5-S1 posterior disc bulges. They inclined more to the right side and seen indenting the theca with partial bilateral foraminal compromise more to the right.
- Normal size and signal of the conus medullaris.
- S1 vertebral body small rounded lesion of low T1 and T2 signal.
- Bilateral L3-4 down to L5-S1 facetar arthropathy.
- No para-spinal soft tissue masses or collection.

OPINION:

- Lumbar spondylosis with multilevel disc bulges; as described.
- S1 vertebral body small lesion of altered marrow signal,?metatstaic, for bone scan correlation.

Dr.Dalia Ibrahim, MD

Dr. Mahmoud ElFekky.

الاسم/ السيد/ خاويح عبد العال
التاريخ/ ٢٠٢٢/٢/٢٠
الأستاذ الدكتور / م. العامر

Duplex study of both LLs arterial & venous system revealed

B Mode:

- The arterial systems including EIA, CFA, PRO, FEMORIS, SFA, PA & its two division's namely anterior & posterior tibial arteries shows normal caliber and echo-free linear vascular appearance with compressible lumen and intact wall, no stenosis, aneurismal dilatation or thrombosis.
- The deep venous systems including EIV, CFV, PRO, FEMORIS, SFV, PV & its tributaries namely anterior & posterior tibial and muscular veins are patent with normal caliber and echo-free linear appearance, compressible lumen and intact wall
- Few inguinal enlarged LNs of non specific pattern

Doppler study:

- Normal triphasic flow pattern of examined arterial tree from EIA down to PTA, ATA and DPA with normal Doppler parameters indices including peak systemic velocities, end diastolic velocities and resistive index
- The venous systems shows normal low-pitched continuous flow pattern with positive distal augmentation, intermittent compression response, respiratory phasicity & valsalva-release signs
- Competent SF&SP junctions

My Thanks.
DR/Said Hussein (MD3).



9/1/2014	التاريخ	هوية عبد العال فرعلى	الاسم
6634/2013	رقم الملف	مسح ثري على العظام	نوع الفحص

Report

^{99m}Tc-MDP Bone Scan

Technique

- Both anterior and posterior whole body scans were acquired at 3 hours after I.V. injection of 740 MBq (20 mCi) of the tracer.

Findings

- Uniform tracer uptake pattern is portrayed all over the surveyed skeleton with no abnormal osseous lesions detected.

Conclusion

- The surveyed skeleton free from metastatic deposits.

Sincerely;

Prof. Dr. Amr Amin

Dr. Hoda Nagui



Patient Name:	hwea Abd el-Aal Farghaly	Patient Gender:	Female
Patient Code:	Q35802	DATE:	3/8/2023 8:59:42 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- 58-year-old female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

Compared to the previous bone study dated 26 July 2022, the current study reveals:

- More or less stationary course of the previously noted osseous lesions involving the right acetabulum.
- Heterogeneous tracer uptake of the spine is noted likely degenerative, for MRI correlation to exclude underlying serious pathology.

Opinion

- Follow- up study reveals more or less stationary course of the previously noted osseous lesion with no newly developed lesions.

Dr. Merna Nasef

Prof Dr. Jehan Younis, MD.

WOMEN IMAGING DEPARTMENT

Patient Name: Haweya abdel aal

Ref. Department:

Date: 23-10-2024

Left sided mammography with complementary bilateral High resolution Ultrasound Examination

Indication: right mastectomy since 2013.

Findings:

- Scattered fibro glandular densities (ACR B).
- No dominant masses, clustered microcalcifications or areas of architectural distortion seen.

Complementary Ultrasound revealed:

- Clear right mastectomy bed and right axilla.
- *Heterogeneous fibro glandular mammary parenchyma with mottled appearance.*
- No solid or cystic lesions.
- Normal contour and skin thickening.
- Normal nipple /areola complexes.
- Left small nonspecific axillary lymph nodes with preserved fatty hilum.

Opinion :-

- *Clear right mastectomy bed.*
- *Left breast fibro adenosis. BIRADS 2.*
- *Annual screening is recommended.*

***Thanks for referral,
Dr. Yasmin Essam Khalifa, MD***



بسم الله الرحمن الرحيم

وحدة دلائل الأورام
قسم الأشعة والجراحة
IMMUNOHISTOCHEMISTRY
٠١٠٦٨٥٢٩٦٨٤ : ت

Patient Name: هوية عبد العال

lab.No:2013/899

Date: 12/2013

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections from paraffin blocks were treated with monoclonal antibodies against ER, PR, Her 2 neu and Ki67 using streptavidin biotin peroxidase system. DAB was used as substrate as well as chromogen. Hematoxylin was used as counter stain

Results:

- Examination of sections treated for ER and PR revealed moderately positive nuclear staining of tumor cells by chromogen.
- Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen
- Examination of sections treated for Ki67 revealed positive nuclear staining in < 14 % of tumor cells by chromogen.

DIAGNOSIS:

- POSITIVE FOR ER AND PR SCORE 4/8
- NEGATIVE FOR HER 2 NEU
- LOW PROLIFERATION INDEX

PROF. DR. SAHAR TALAAT

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 5172

Age: 49 Sex: female

Name: هبة عبد العال فرغلى

Clinical diagnosis: BREAST MASS

Hospital_no: 14164

Nature of specimen: COMPLETION MASTECTOMY

Department: 27a

Date: 04/11/2013

PATHOLOGY REPORT

GROSS:

Mastectomy specimen with overlying skin ellipse measures 14x4 cm showing a sutured wound measurers 2.5 cm long. Sections revealed firm greyish area measures 1x1 cm surrounding the sutures situated 1 cm from skin & 2 cm from deep margin. Another greyish area, rubbery, measures 0.5x0.5 cm was detected.

Microscopic:

Examination of the specimen received revealed fibrosis, fat necrosis & giant cells at site of previous resection of the tumor.

Rest of breast tissue shows an area of fibroadenosis with ductal atypical epithelial hyperplasia & focal solid intraductal carcinoma.

Nipple & deep margins are free.

No evidence of invasive malignancy.

DIAGNOSIS:

* COMPLETION MASTECTOMY:

* FOCAL SOLID INTRADUCT CARCINOMA (<5%)

* FIBROADENOSIS WITH ATYPICAL EPITHELIAL HYPERPLASIA

* FAT NECROSIS

* FREE NIPPLE & DEEP MARGINS.

PROF. DR. SAHAR TALAAT
DR. MONA SALAH

د. سحر تالات
د. منة صلاح



Name: HAWYA ABDELAAL FARGHALI	Date: 4/14/2025 2:27:50 PM
Age:	ID: 500868

Post-contrast CT scan of the abdomen and pelvis revealed:

****Reported history of breast cancer showing**

- Average sized liver showing homogenous texture. No focal lesions or signs of intrahepatic biliary radicle dilatation.
- Gall bladder is averagely distended
- Normal CT appearance of the spleen, showing homogenous texture. No focal lesions seen with small splenule close to its upper pole.
- Normal form and size of the suprarenal glands, no retroperitoneal masses.
- Normal site and size of both kidneys. No renal stones, masses or back pressure changes.
- No pancreatic masses, also no dilatation of the pancreatic duct.
- No sizable abdominal, retroperitoneal or pelvic lymphadenopathy.
- No ascites.
- Normal pelvic organs
- Well distended urinary bladder showing regular outline, normal wall thickness, with no masses or diverticulum formation.
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Dr. Asmaa Yahia

Dr. Marwa Shaker, MD.



Patient No:	hwen Abd el-Aal Farghaly Hamada	Patient Gender:	Female
Patient ID:	Q44392	DATE:	3/25/2025 2:03:02 PM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

Compared to the previous study dated 30/09/2024 (only report is available but no films are available), the current study reveals:

- ✓ Newly developed osseous lesion in the sternum.
- ✓ Stationary course of the previously noted osseous tracer uptake related to the skull and DV7, LV1.
- ✓ Progressive course of the previously noted osseous tracer uptake related to right acetabulum and greater trochanter.
- ✓ Regressive course of the previously noted tracer uptake related to the right iliac bone.
- ✓ Still non-visualized right kidney for radiological correlation.

Opinion

- ✓ Follow-up study showing newly developed lesion in the sternum with mixed response of the previously seen osseous lesions, for follow up.

Dr. Eyad Hesham

Dr. Nahla Dessoki, MD



Name: HAWYA ABDELAAL FARGHALI	Date: 4/7/2025
Age:	ID: 500868

THORACIC IMAGING UNIT

CT SCAN OF THE CHEST REVEALED:

- Evidence of right mastectomy.
- No pulmonary consolidation or cavitation.
- Right lower lobe few tiny 2 mm pulmonary nodules.
- Subcentimetric mediastinal, right hilar and left axillary lymph nodes.
- No pleural or pericardial sac collections.
- No gross cardiomegaly.
- **Scattered vertebral and sternal heterogenous osseous lesions, for bone scan correlation.**

Opinion:

- No current CT evidence of pneumonia.
- Right lower lobe few tiny pulmonary nodules, for follow-up.

Dr. Moustafa Khairy, MD

Patient Name: HAWEAA A.ELAAL FARGARY	Patient ID: 26402192500868
Study Date: 26/01/2022	Age: 57 Years

**left Sided Full Field Mammography with Complementary Ultrasound
Examinations Revealed**

Indications:

- Right mastectomy

Findings:

- Scattered fibroglandular densities (ACR'b').
- No spiculated mass lesions or microcalcific clusters seen.
- **Complementary Ultrasound** showed no underlying solid or cystic lesions.
- Examination of the operative bed showed no residual or recurrent mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex.
- left non specific axillary lymphadenopathy.
- no supracavicular lymphadenopathy

Opinion:

- Clear operative bed (BIRADS 2)
- Normal sono mammography study of the left breast (BIRADS 1).

Thanks for reference

Dr. Nourhan Mohamed



د. سحر تالات

وحدة دلائل الأورام
IMMUNOHISTOCHEMISTRY
ت : ٠١٠٦٨٥٢٩٦٨٤

Patient Name: هوية عبد العال

lab.No: 2013/899

Date: 12/2013

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections from paraffin blocks were treated with monoclonal antibodies against ER, PR, Her 2 neu and Ki67 using streptavidin biotin peroxidase system. DAB was used as substrate as well as chromogen. Hematoxylin was used as counter stain

Results:

- Examination of sections treated for ER and PR revealed moderately positive nuclear staining of tumor cells by chromogen.
- Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen
- Examination of sections treated for Ki67 revealed positive nuclear staining in < 14 % of tumor cells by chromogen.

DIAGNOSIS:

- POSITIVE FOR ER AND PR SCORE 4/8
- NEGATIVE FOR HER 2 NEU
- LOW PROLIFERATION INDEX

PROF. DR. SAHAR TALAAT

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 5172 Age: 49 Sex: female
Name هويدا عبد العال فرحاني
Clinical diagnosis BREAST MASS Hospital_no: 14164
Nature of specimen COMPLETION MASTECTOMY
Department: 27a Date: 04/11/2013

PATHOLOGY REPORT

GROSS:

Mastectomy specimen with overlying skin ellipse measures 14x4 cm showing a sutured wound measurers 2.5 cm long. Sections revealed firm greyish area measures 1x1 cm surrounding the sutures situated 1 cm from skin & 2 cm from deep margin. Another greyish area, rubbery, measures 0.5x0.5 cm was detected.

Microscopic:

Examination of the specimen received revealed fibrosis, fat necrosis & giant cells at site of previous resection of the tumor.

Rest of breast tissue shows an area of fibroadenosis with ductal atypical epithelial hyperplasia & focal solid intraductal carcinoma.

Nipple & deep margins are free.

No evidence of invasive malignancy.

DIAGNOSIS:

- * COMPLETION MASTECTOMY :
- * FOCAL SOLID INTRADUCT CARCINOMA (<5%)
- * FIBROADENOSIS WITH ATYPICAL EPITHELIAL HYPERPLASIA.
- * FAT NECROSIS.
- * FREE NIPPLE & DEEP MARGINS.

PROF. DR. SAHAR TALAAT
DR. MONA SALAH

د. سحر تالات
د. منة صلاح



Presidential Program for Women's Health
Ministry of Health and Wellness

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والرفاهية



MDT form

A-Patient ID:

1- Name : هوية عبدالله فرطى حماد

Sex:

3- Medical number:

Female

83274987

Old Med :

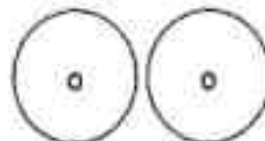
Old Med

1309-2025

2-ID number: 264011C2500868

Patient Age : 61 Y

Y



Family history

Family history

Phone

Phone

01221292933

BMI

BMI

REASON OF MDT

REASON OF MDT

marital status

marital status

NRS Score

NRS

GROUP

GROUP

Date of MDT

Date of MDT

2025-05-18

☐

Co morbidity

Co morbidity

Menopausal status

Menopausal status

Initial STAGE

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :

BX

ER

PR

Her 2 Neu

SISH

Ki67

EXCISE

EXCISE

EXCISE

EXCISE

EXCISE

EXCISE

T

N

M

BONE

Sonomammography

PATIENT HISTORY

breast cancer since 2013-->lumpectomy -->completion mastectomy-->DCIS
adj chemo AC x4-->4 taxanes-->PORT-->adj endocrine x 9 yrs end on 2022
bony mets 2024-->AI-->now DP (only bone)-->palliative RTH

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

BONE SCAN ONLY COMPARED WITHOUT IMAGES

OLD BIOPSY NOT AVAILABLE

FOR BONE SCAN AFTER 3 MONTH WITH COMPARISON LAST STUDY

KEEP ON ai , ZOMETA/3 MONTH TILL RESULT

SIGNATURE

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

RADIOLOGY

RADIOLOGY

SURGERY

SURGERY

PATHOLOGY

PATHOLOGY

Psychiatric

Psychiatric

Nutrition

Nutrition



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
مصر
13-9
C. 50

MDT FORM

/ / 20

G

Name	حور نبال	MRN	13574917
Age	61	ID	
Menopausal status	post menopause	Phone	0155199933
NRS SCORE		Marital status	1577049134
BMI		Family history	
CO morbidity	-ve	REASON OF MDT	

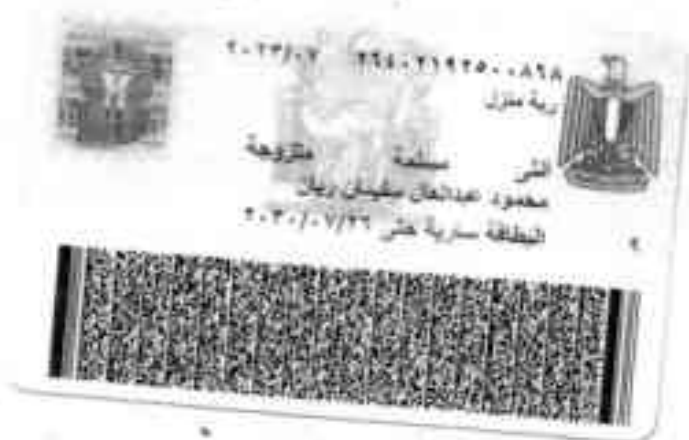
Clinical Data

BX:	R	L
ER: 4/8	PR: 4/8	HER 2: -ve
Ki67:		
Sonomammography:	TNM	
Mets Workup:	known case of breast cancer since 2013	
	→ lumpectomy (path report not avail)	
	→ completion mastectomy → DCIS	

Recommendations:	Adl chemo. 4 AC → 4 Taxanes
	PORT
	Adl endocrine x 9 years end on 2022
	Bony mets 2024 → AI

MDT OUTCOME & PLAN	now Dp (only bone) → Pathologic RT
	for faspdex + CDK4/6

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric





Name: هوبة عبد العال Age: 60 Ys Date: 17 August 2024

Pelvi-abdomenal U/S study, revealed

- Liver is average in size, measuring about 13.8 cm, with smooth outline & diffuse bright homogenous echopattern. No focal lesions were detected.
- No intra or extra hepatic biliary radicles dilatation.
- Gall bladder: is normal capacity and wall thickness. No stones or mud inside.
- Portal vein is patent & measures (9.6 mm) and C.B.D (0.4 mm).
- Spleen normal shape & measures (8.8 cm) and homogenous echogenicity. No focal.
- Pancreas & Para-Aortic area: normal U/S appearance.
- Both kidneys: They are of normal in site, size, shape, parenchymal thickness and echogenicity with no renal stones, no back pressure obstructive changes.
RK = 10.3 X 3.7 cm with parenchymal thickness = 1.5 cm
LK = 10.4 X 3.7 cm with parenchymal thickness = 1.0 cm
- Urinary bladder is empty at time of examination.
- Uterus: is of average size & homogenous myometrium.
- Adenexa: no gross focal lesions.
- No ascites or pelvic collection.
- Prominent colonic gases.

Conclusion:

- ***Bright liver (?? Fatty infiltration).***
- ***Prominent colonic gaseous distention.***

Much obliged, yours,

Akram Hafez

Radiodiagnosis Specialist

Patient name: Hawea Abd Alaal

Study Date: 22/10/2022

CT scan of the abdomen & pelvis revealed:

****Reported case of invasive right breast cancer.**

****The examination is partially degraded by motion artifact, absence of adequate thin cutting and triphasic post contrast enhancement.**

- Average hepatic size showing homogenous parenchymal pattern. No focal hepatic lesions or biliary radicles dilatation.
- **Relatively bulky left suprarenal gland.**
- Normal CT appearance of the spleen, pancreas, right suprarenal gland, IVC and urinary bladder. **Splenule is noted anterior to the spleen.**
- Normal appearance of both kidneys with no gross backpressure changes or radiodense calculi.
- **Aortic calcified atheromatous plaques are noted.**
- **Few enlarged paraaortic, mesenteric and aorto-caval lymph nodes, the largest is at the latter measuring about 1.2x0.6 cm in maximum axial dimensions.**
- No ascites or pelvic collections.
- **Few pelvic phleboliths are also noted.**

Dr.Aya Fawzy, MSc.



Name: HAWYA ABDELAAL FARGHALI	Date: 4/7/2025
Age:	ID: 500868

THORACIC IMAGING UNIT

CT SCAN OF THE CHEST REVEALED:-

- Evidence of right mastectomy.
- No pulmonary consolidation or cavitation.
- Right lower lobe few tiny 2 mm pulmonary nodules.
- Subcentimetric mediastinal, right hilar and left axillary lymph nodes.
- No pleural or pericardial sac collections.
- No gross cardiomegaly.
- **Scattered vertebral and sternal heterogenous osseous lesions, for bone scan correlation.**

Opinion:

- No current CT evidence of pneumonia.
- Right lower lobe few tiny pulmonary nodules, for follow-up.

Dr. Moustafa Khairy, MD



Name: HAWYA ABDELAAL FARGHALI	Date: 4/14/2025 2:27:50 PM
Age:	ID: 500868

Post-contrast CT scan of the abdomen and pelvis revealed:

****Reported history of breast cancer showing**

- Average sized liver showing homogenous texture. No focal lesions or signs of intrahepatic biliary radicle dilatation.
- Gall bladder is averagely distended
- Normal CT appearance of the spleen, showing homogenous texture. No focal lesions seen with small splenule close to its upper pole.
- Normal form and size of the suprarenal glands, no retroperitoneal masses.
- Normal site and size of both kidneys. No renal stones, masses or back pressure changes.
- No pancreatic masses, also no dilatation of the pancreatic duct.
- No sizable abdominal, retroperitoneal or pelvic lymphadenopathy.
- No ascites.
- Normal pelvic organs
- Well distended urinary bladder showing regular outline, normal wall thickness, with no masses or diverticulum formation.
- Right acetabular and right femoral head heterogeneous focal sclerosis ? metastatic for bone scan correlation

Dr. Asmaa Yahia

Dr. Marwa Shaker, MD.



AL SHROOK SCAN

الشروق للأشعة

15184

Pat.Name: هويدا عبدالعالم لمرغش

Pat.ID: 442526

Date: 13/08/2024

Pat. Age 60 Year(s)

Ref.Prof.Doc: Mohamed Saad Al Emam

MRI OF THE PELVIS & BOTH HIPS

TECHNIQUE:

- Sagittal T2WIs
- Axial T1 and T2 WI
- Coronal T1, T2 and STIR WI

FINDINGS:

- Multiple variable sized extensive infiltrative marrow lesions are seen involving the right hemi pelvis, right femoral head and neck. The most sizable lesion involves most of the right iliac blade extending inferiorly to the acetabulum, ischium, superior and inferior pubic rami. It appears associated with minimal extra osseous soft tissue rim on non contrast basis. Other smaller lesions are seen at the right sacral alum as well as the right femoral head and neck. Fewer smaller similar lesions affecting the iliac bone and acetabulum. All these lesions display low signal in T1 WIs and bright signal in T2 and STIR WIs.
- Intact both hip joints with no joint effusion or synovial thickening.
- Normal signal and comparable bulk of the different muscle groups around both hip joints.
- Preserved fascial fat planes.

OPINION:

- Multiple extensive infiltrative marrow lesions involving the sacrum, iliac bones and right femur impressive of metastatic deposits.

Dr. Dalia Ibrahim, MD

Dr. Mahmoud ElFekky.



AL SHROOK SCAN

الشروق للأشعة

15184

Pat.Name: موديا عبدالعزى

Pat.ID: 442526

Date: 13/08/2024

Pat. Age 60 Year(s)

Ref.Prof.Doc: Mohamed Saad Al Emam

MRI OF THE LUMBOSACRAL SPINE

TECHNIQUE:

- Sagittal T1 and T2 WIs
- Axial T2 WIs

FINDINGS:

- Straightened lumbar lordosis.
- Normal dimensions of the spinal canal.
- Mild lumbar spondylosis evident by small marginal osteophytic lipping of the vertebral endplates and degeneration of the lumbar discs.
- L3-4, L4-5 and L5-S1 posterior disc bulges. They inclined more to the right side and seen indenting the theca with partial bilateral foraminal compromise more to the right.
- Normal size and signal of the conus medullaris.
- S1 vertebral body small rounded lesion of low T1 and T2 signal.
- Bilateral L3-4 down to L5-S1 facetar arthropathy.
- No para-spinal soft tissue masses or collection.

OPINION:

- Lumbar spondylosis with multilevel disc bulges; as described.
- S1 vertebral body small lesion of altered marrow signal,?metatstaic, for bone scan correlation.

Dr.Dalia Ibrahim, MD

Dr. Mahmoud ElFekky.

الاسم/ السيد/ خاويح عبد العال
التاريخ/ ٢٠٢٢/٢/٢٠
الأستاذ الدكتور / م. العامر

Duplex study of both LLs arterial & venous system revealed

B Mode:

- The arterial systems including EIA, CFA, PRO, FEMORIS, SFA, PA & its two division's namely anterior & posterior tibial arteries shows normal caliber and echo-free linear vascular appearance with compressible lumen and intact wall, no stenosis, aneurismal dilatation or thrombosis.
- The deep venous systems including EIV, CFV, PRO, FEMORIS, SFV, PV & its tributaries namely anterior & posterior tibial and muscular veins are patent with normal caliber and echo-free linear appearance, compressible lumen and intact wall
- Few inguinal enlarged LNs of non specific pattern

Doppler study:

- Normal triphasic flow pattern of examined arterial tree from EIA down to PTA, ATA and DPA with normal Doppler parameters indices including peak systemic velocities, end diastolic velocities and resistive index
- The venous systems shows normal low-pitched continuous flow pattern with positive distal augmentation, intermittent compression response, respiratory phasicity & valsalva-release signs
- Competent SF&SP junctions

My Thanks,
DR/Said Hussein (MD3).



9/1/2014	التاريخ	هوية عبد العال فرعلى	الاسم
6634/2013	رقم الملف	مسح ثري على العظام	نوع الفحص

Report

^{99m}Tc-MDP Bone Scan

Technique

- Both anterior and posterior whole body scans were acquired at 3 hours after I.V. injection of 740 MBq (20 mCi) of the tracer.

Findings

- Uniform tracer uptake pattern is portrayed all over the surveyed skeleton with no abnormal osseous lesions detected.

Conclusion

- The surveyed skeleton free from metastatic deposits.

Sincerely;

Prof. Dr. Amr Amin

Dr. Hoda Nagui



Patient Name:	hwea Abd el-Aal Farghaly	Patient Gender:	Female
Patient Code:	Q35802	DATE:	3/8/2023 8:59:42 AM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- 58-year-old female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

Compared to the previous bone study dated 26 July 2022, the current study reveals:

- More or less stationary course of the previously noted osseous lesions involving the right acetabulum.
- Heterogeneous tracer uptake of the spine is noted likely degenerative, for MRI correlation to exclude underlying serious pathology.

Opinion

- Follow- up study reveals more or less stationary course of the previously noted osseous lesion with no newly developed lesions.

Dr. Merna Nasef

Prof Dr. Jehan Younis, MD.

WOMEN IMAGING DEPARTMENT

Patient Name: Haweya abdel aal

Ref. Department:

Date: 23-10-2024

Left sided mammography with complementary bilateral High resolution Ultrasound Examination

Indication: right mastectomy since 2013.

Findings:

- Scattered fibro glandular densities (ACR B).
- No dominant masses, clustered microcalcifications or areas of architectural distortion seen.

Complementary Ultrasound revealed:

- Clear right mastectomy bed and right axilla.
- *Heterogeneous fibro glandular mammary parenchyma with mottled appearance.*
- No solid or cystic lesions.
- Normal contour and skin thickening.
- Normal nipple /areola complexes.
- Left small nonspecific axillary lymph nodes with preserved fatty hilum.

Opinion :-

- *Clear right mastectomy bed.*
- *Left breast fibro adenosis. BIRADS 2.*
- *Annual screening is recommended.*

***Thanks for referral,
Dr. Yasmin Essam Khalifa, MD***



بسم الله الرحمن الرحيم

وحدة دلائل الأورام
قسم الأشعة والجراحة
IMMUNOHISTOCHEMISTRY
٠١٠٦٨٥٢٩٦٨٤ : ت

Patient Name: هوية عبد العال

lab.No:2013/899

Date: 12/2013

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

Procedure:

Sections from paraffin blocks were treated with monoclonal antibodies against ER, PR, Her 2 neu and Ki67 using streptavidin biotin peroxidase system. DAB was used as substrate as well as chromogen. Hematoxylin was used as counter stain

Results:

- Examination of sections treated for ER and PR revealed moderately positive nuclear staining of tumor cells by chromogen.
- Examination of sections treated for Her 2 neu revealed no membranous staining of tumor cells by chromogen
- Examination of sections treated for Ki67 revealed positive nuclear staining in < 14 % of tumor cells by chromogen.

DIAGNOSIS:

- POSITIVE FOR ER AND PR SCORE 4/8
- NEGATIVE FOR HER 2 NEU
- LOW PROLIFERATION INDEX

PROF. DR. SAHAR TALAAT

CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT

Pathology No: 5172

Age: 49 Sex: female

Name: هبة عبد العال فرغلى

Clinical diagnosis: BREAST MASS

Hospital_no: 14164

Nature of specimen: COMPLETION MASTETCOMY

Department: 27a

Date: 04/11/2013

PATHOLOGY REPORT

GROSS:

Mastectomy specimen with overlying skin ellipse measures 14x4 cm showing a sutured wound measurers 2.5 cm long. Sections revealed firm greyish area measures 1x1 cm surrounding the sutures situated 1 cm from skin & 2 cm from deep margin. Another greyish area, rubbery, measures 0.5x0.5 cm was detected.

Microscopic:

Examination of the specimen received revealed fibrosis, fat necrosis & giant cells at site of previous resection of the tumor.

Rest of breast tissue shows an area of fibroadenosis with ductal atypical epithelial hyperplasia & focal solid intraductal carcinoma.

Nipple & deep margins are free.

No evidence of invasive malignancy.

DIAGNOSIS:

* COMPLETION MASTETCOMY :

* FOCAL SOLID INTRADUCT CARCINOMA (<5%)

* FIBROADENOSIS WITH ATYPICAL EPITHELIAL HYPERPLASIA.

* FAT NECROSIS.

* FREE NIPPLE & DEEP MARGINS.

PROF. DR. SAHAR TALAAT
DR. MONA SALAH

د. سحر تالات
د. منة صلاح



Name: HAWYA ABDELAAL FARGHALI	Date: 4/14/2025 2:27:50 PM
Age:	ID: 500868

Post-contrast CT scan of the abdomen and pelvis revealed:

****Reported history of breast cancer showing**

- Average sized liver showing homogenous texture. No focal lesions or signs of intrahepatic biliary radicle dilatation.
- Gall bladder is averagely distended
- Normal CT appearance of the spleen, showing homogenous texture. No focal lesions seen with small splenule close to its upper pole.
- Normal form and size of the suprarenal glands, no retroperitoneal masses.
- Normal site and size of both kidneys. No renal stones, masses or back pressure changes.
- No pancreatic masses, also no dilatation of the pancreatic duct.
- No sizable abdominal, retroperitoneal or pelvic lymphadenopathy.
- No ascites.
- Normal pelvic organs
- Well distended urinary bladder showing regular outline, normal wall thickness, with no masses or diverticulum formation.
- Right acetabular and right femoral head heterogeneous focal sclerosis ? metastatic for bone scan correlation

Dr. Asmaa Yahia

Dr. Marwa Shaker, MD.



Patient No:	hwen Abd el-Aal Farghaly Hamada	Patient Gender:	Female
Patient ID:	Q44392	DATE:	3/25/2025 2:03:02 PM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- Female patient with history of right breast cancer.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

Compared to the previous study dated 30/09/2024 (only report is available but no films are available), the current study reveals:

- ✓ Newly developed osseous lesion in the sternum.
- ✓ Stationary course of the previously noted osseous tracer uptake related to the skull and DV7, LV1.
- ✓ Progressive course of the previously noted osseous tracer uptake related to right acetabulum and greater trochanter.
- ✓ Regressive course of the previously noted tracer uptake related to the right iliac bone.
- ✓ Still non-visualized right kidney for radiological correlation.

Opinion

- ✓ Follow-up study showing newly developed lesion in the sternum with mixed response of the previously seen osseous lesions, for follow up.

Dr. Eyad Hesham

Dr. Nahla Dessoki, MD



Name: HAWYA ABDELAAL FARGHALI	Date: 4/7/2025
Age:	ID: 500868

THORACIC IMAGING UNIT

CT SCAN OF THE CHEST REVEALED:

- Evidence of right mastectomy.
- No pulmonary consolidation or cavitation.
- Right lower lobe few tiny 2 mm pulmonary nodules.
- Subcentimetric mediastinal, right hilar and left axillary lymph nodes.
- No pleural or pericardial sac collections.
- No gross cardiomegaly.
- **Scattered vertebral and sternal heterogenous osseous lesions, for bone scan correlation.**

Opinion:

- No current CT evidence of pneumonia.
- Right lower lobe few tiny pulmonary nodules, for follow-up.

Dr. Moustafa Khairy, MD

Patient Name: HAWEAA A.ELAAL FARGARY	Patient ID: 26402192500868
Study Date: 26/01/2022	Age: 57 Years

**left Sided Full Field Mammography with Complementary Ultrasound
Examinations Revealed**

Indications:

- Right mastectomy

Findings:

- Scattered fibroglandular densities (ACR'b').
- No spiculated mass lesions or microcalcific clusters seen.
- **Complementary Ultrasound** showed no underlying solid or cystic lesions.
- Examination of the operative bed showed no residual or recurrent mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex.
- left non specific axillary lymphadenopathy.
- no supracavicular lymphadenopathy

Opinion:

- Clear operative bed (BIRADS 2)
- Normal sono mammography study of the left breast (BIRADS 1).

Thanks for reference

Dr. Nourhan Mohamed



Dr. Sahar Talaat

وحدة دلائل الأورام
IMMUNOHISTOCHEMISTRY
ت : ٠١٠٦٨٥٢٩٦٨٤

Patient Name: هوية عبد العال

lab.No: 2013/899

Date: 12/2013

Sex: Female

Markers: ER, PR, Her 2 neu and Ki67 .

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Sections from paraffin blocks were treated with monoclonal antibodies against ER, PR, Her 2 neu and Ki67 using streptavidin biotin peroxidase system. DAB was used as substrate as well as chromogen. Hematoxylin was used as counter stain

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DIAGNOSIS:

- POSITIVE FOR ER AND PR SCORE 4/8
- NEGATIVE FOR HER 2 NEU
- LOW PROLIFERATION INDEX

PROF. DR. SAHAR TALAAT

**CAIRO UNIVERSITY
FACULTY OF MEDICINE
PATHOLOGY DEPARTMENT**

Pathology No: 5172 Age: 49 Sex: female
Name هويدا عبد العال فرغلي
Clinical diagnosis BREAST MASS Hospital_no: 14164
Nature of specimen COMPLETION MASTECTOMY
Department: 27a Date: 04/11/2013

PATHOLOGY REPORT

GROSS:

Mastectomy specimen with overlying skin ellipse measures 14x4 cm showing a sutured wound measurers 2.5 cm long. Sections revealed firm greyish area measures 1x1 cm surrounding the sutures situated 1 cm from skin & 2 cm from deep margin. Another greyish area, rubbery, measures 0.5x0.5 cm was detected.

Microscopic:

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Rest of breast tissue shows an area of fibroadenosis with ductal atypical epithelial hyperplasia & focal solid intraductal carcinoma.

Nipple & deep margins are free.

No evidence of invasive malignancy.

DIAGNOSIS:

- * COMPLETION MASTECTOMY :
- * FOCAL SOLID INTRADUCT CARCINOMA (<5%)
- * FIBROADENOSIS WITH ATYPICAL EPITHELIAL HYPERPLASIA.
- * FAT NECROSIS.
- * FREE NIPPLE & DEEP MARGINS.

PROF. DR. SAHAR TALAAT
DR. MONA SALAH

د. سحر تالات
د. منى صلاح



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الاستشارة
مصر
13-9
C. 50

MDT FORM

/ / 20

G

Name	حور نبال	MRN	13574917
Age	61	ID	
Menopausal status	post menopause	Phone	0155199933
NRS SCORE		Marital status	1577049134
BMI		Family history	
CO morbidity	-ve	REASON OF MDT	

Clinical Data

BX:	R	L
ER: 4/8	PR: 4/8	HER 2: -ve
Ki67:		
Sonomammography:	TNM	
Mets Workup:	known case of breast cancer since 2013	
	→ lumpectomy (path report not avail)	
	→ completion mastectomy → DCIS	

Recommendations:	Adl chemo. 4 AC → 4 Taxanes
	PORT
	Adl endocrine x 9 years end on 2022
	Bony mets 2024 → AI

MDT OUTCOME & PLAN	now Dp (only bone) → Pathologic RT
	for faspdex + CDK4/6

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric